

Review test results and manage accordingly:

Assess	Note
TB microscopy (smear) and culture	If month 4 smear/culture positive or smear/culture becomes positive after being negative: assess adherence ➔ 173, review all previous sputum results and request LPA and extended phenotypic DST on latest culture positive specimen. Present to NCAC as soon as possible to advise on rescue regimen. Consider referral for surgical assessment.
LPA and DST results (drug susceptibility)	1st and 2nd line LPA will be done when reflex DST testing is requested at diagnosis: - If LPA is sensitive to INH, INH phenotypic DST will be automatically tested by laboratory. - If LPA is sensitive to fluoroquinolones, fluoroquinolone phenotypic DST will be automatically tested by laboratory. - If LPA is resistant to fluoroquinolones or injectables, or both inhA and katG mutations present, 2nd line phenotypic DST will be automatically tested by laboratory.
Chest x-ray	If chest x-ray worse despite treatment, discuss with specialist.
ECG	Calculate QTcF¹. If ≥ 450ms: - If symptoms (chest pain, palpitations, dizziness or faintness), discuss with specialist or refer to hospital same day. - If no symptoms: - If QTcF < 470ms: continue treatment and routine ECG monitoring. - If QTcF 470-499ms: repeat ECG at rest same day to confirm. Check potassium, magnesium and TSH and treat if abnormal. Check for medications that prolong QT interval² and discuss with experienced TB doctor or specialist. Repeat ECG weekly until < 470ms. - If QTcF ≥ 500ms: repeat ECG at rest same day to confirm. Check potassium, magnesium and TSH and treat if abnormal. Stop all medications that prolong QT interval² including RR-TB medications (moxifloxacin, delamanid, clofazimine, bedaquiline). Discuss with experienced TB doctor or specialist same day.
Audiometry (hearing test)	If on amikacin and any changes to hearing, stop amikacin and discuss possible medication substitutions ³ with PCAC/NCAC.
Vision	If any change in visual acuity, stop linezolid and ethambutol and refer to eye specialist same day. Discuss possible medication substitutions ³ with PCAC/NCAC.
Pregnancy test	If pregnant ➔ 159 and present to NCAC. Avoid delaying treatment, start while awaiting response.
Glucose	If known diabetes, assess glucose control more often ➔ 130. If not known with diabetes, check glucose ➔ 17.
HIV	If HIV positive, give routine care ➔ 111. If on ART, check if ART regimen needs adjusting ➔ 101. If not on ART, start or re-restart ➔ 101.
FBC and differential count	If Hb < 8 , neutrophils < 0.75 or platelets < 50 , stop linezolid and discuss with PCAC/NCAC or refer for admission.
ALT	- If ALT ≥ 200 or jaundice, stop all medications and refer same day. - If ALT 50-199: if symptoms (nausea/vomiting/abdominal pain) ➔ 105. - If no symptoms: continue medications and monitor for symptoms. If ALT 120-199, also repeat ALT weekly until < 120.
Creatinine (eGFR)	If eGFR ≤ 50 , avoid amikacin. If on amikacin, stop amikacin and discuss possible medication substitutions ³ with PCAC/NCAC.
Potassium	- If potassium ≤ 2.5, refer same day. - If potassium 2.6-3.5, do ECG: - If any arrhythmia on ECG or if patient has muscle weakness or vomiting, refer same day. - If neither, give potassium chloride 2 tablets 12 hourly and repeat potassium within 1 week. Manage again according to result.
Magnesium	If magnesium < 0.6 , give magnesium chloride 500-1000mg orally 12 hourly for 1 month. If < 0.4 , refer for IV magnesium.
TSH (thyroid function)	If TSH raised, check FT4. If FT4 low, hypothyroidism likely: - Give levothyroxine 100mcg daily and repeat TSH and FT4 after 2 months, unless: - If ≥ 60 years: give instead levothyroxine 50mcg daily and repeat TSH and FT4 after 1 month. - If known ischaemic heart disease: give instead levothyroxine 25mcg daily and repeat TSH and FT4 after 1 month. - If repeat FT4 still low, increase levothyroxine by 25mcg every 4 weeks until FT4 within normal range. - Once RR-TB treatment completed, continue levothyroxine for 2-3 months, then wean while continuing to monitor TSH and FT4.

Continue to review results ➔ 101.

¹QTcF is QT interval corrected for heart rate: online calculator (Fridericia's formula) can be accessed via <https://www.mdcalc.com/corrected-qt-interval-qtcf> or calculate manually: $QTcF = QT / (60 / \text{heart rate})^{0.33}$. ²Medications that may prolong QT interval include: anti-arrhythmics (e.g amiodarone), psychotropics (e.g haloperidol), macrolide antibiotics (e.g erythromycin, azithromycin, clarithromycin), fluoroquinolone antibiotics (e.g ciprofloxacin, levofloxacin, moxifloxacin) and antifungal drugs (e.g fluconazole, ketoconazole). ³Continue other medications while awaiting response from PCAC/NCAC.